

Every pregnant woman and every newborn in India is in scope

This does not serve all 147 crore Indians, and we do not claim it does. Today's scope is maternal and newborn care, so the people it reaches are **mothers and their children** — about **2.3 crore births a year**, and the women carrying them. An ASHA is the first and often the only health worker present at the home for most of those births. Maternal mortality is 88 per lakh live births (SRS 2021–23) and neonatal mortality 19 per 1,000 (SRS 2023): **roughly 20,000 mothers and 4.4 lakh newborns a year**. Most newborn deaths fall in the first 48 hours — exactly when an ASHA is in the home, the decision is hers, and her only reference is a book she cannot search. That is the scope today, and it is also the entry point: the same device at the same ₹0 per question is how basic medical knowledge eventually reaches every Indian in their own language.

2.3 cr BIRTHS A YEAR, ALL IN AN ASHA'S CATCHMENT	~4.6 cr MOTHERS AND NEWBORNS IN SCOPE EACH YEAR	10.2 lakh ASHA WORKERS UNDER NHM	₹0 MARGINAL COST PER QUESTION ASKED
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01 Team

Sueb Sabugar ENGINEERING Built the on-device stack end to end — speech, translation, retrieval, language model, and the memory work behind it.	Saleha Shaikh HEALTH CONTENT & VALIDATION Owns the knowledge base, the scored question set, and review of every answer it may give.	Mubeen Pothigara DEPLOYMENT & PARTNERSHIPS Runs pilot logistics and the district health and NGO relationships a deployment needs.
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02 Target market

Three parties, deliberately distinct: **NHM or the district health society buys, the ASHA uses, the mother and newborn benefit**. About 9 in 10 of the 10.2 lakh ASHAs work rural blocks, where a cloud assistant is not an option at all.

It assists the ASHA; it replaces neither her nor what is already deployed. Frontline apps such as Gujarat's TeCHO+ schedule the visit and capture the data — they tell her who to see. This answers the clinical question at the doorstep: what to do once she is there.

One device covers all of India. Each unit carries one language pack, and BHASHINI already publishes ASR, translation and speech for every scheduled language. A new state is a model swap on the same hardware — not a new build.

03 Revenue model

Government procurement through NHM. ASHAs are an existing cadre with a budget line for kits and training; the device is sold into that line, in two parts:

- Device sale** — one-time, per unit, to the state or district health society.
- Annual content & support** — revisions, new protocols, replacement. Recurring, and what keeps answers correct as guidelines change. NGO and CSR-funded deployments are the same product on a different purchase order — how the first pilots get paid for.

04 Cost structure

LINE	TODAY	AT VOLUME	WHAT DRIVES IT
Hardware per unit	₹46,482	₹12,000	Today it is a developer kit. The stack needs 4.2 GB and 5.1 W and nothing is CUDA-only, so a commodity 8 GB ARM board is a port, not a rewrite.
Cloud inference, connectivity, model licensing	₹0	₹0	All three are structurally zero. A cloud assistant pays per query forever; this one does not. There is no SIM, no data plan and nothing to reimburse — which also removes the largest cause of field failure. Weights are open throughout, with one exception to close before sale: the Hindi voice is CC-BY-NC-SA and must be swapped.
Content & support / unit / yr	—	₹800	Revisions, protocol updates, support, replacement. Scales with units, not questions.

05 Path to scale

Three things scale independently on the same device — **what it knows, what it can sense, what language it speaks**. None needs new hardware: the production carrier already carries a camera interface (populating it is a ₹900 option) and the language packs already exist at BHASHINI. The camera path is gated on approval, not on our own confidence — we do not intend to choose the conditions ourselves, and nothing visual ships as a health claim until it clears the Ministry of Health and ICMR route. It will always present a reason to refer, never a result.

AXIS	PHASE 1 · 0–6 MONTHS	PHASE 2 · 6–18 MONTHS	PHASE 3 · 18–36 MONTHS
Knowledge	All seven ASHA training modules (Book No. 1–7) on the device, not the single module the prototype carries today.	State protocols, immunisation schedules, and NHM's yearly revisions.	General primary-care reference — what any household asks.
Capability	Voice question, voice answer, refusal when the manual does not cover it.	Camera screening trial under MoH / ICMR protocol, starting with neonatal jaundice — smartphone screening reached 100% sensitivity against serum bilirubin in a 546-neonate study (JAMA Netw Open, 2024).	Whichever screening aids the Ministry prioritises. Jaundice is our first candidate, not the limit.
Language	Hindi — the language every accuracy number here was measured in.	Two more BHASHINI languages, proving the swap is configuration.	Any scheduled language on request; the application does not change.
Deployment	25 devices, one district, one full quarter of real use.	2,000–5,000 units on an NHM Programme Implementation Plan line.	50,000+ units, multi-state, at ₹12,000 per unit.